

ORDER SHEET
IN THE LAHORE HIGH COURT, MULTAN BENCH, MULTAN.
(JUDICIAL DEPARTMENT)

1. Case No. **Crl. Misc. No. 10010-B/2024**
Muhammad Ramzan **Versus** *The State etc.*

2. Case No. **Crl. Misc. No. 1060-B/2025**
Jahangir **Versus** *The State etc.*

Sr.No.of order/ Proceedings	Date of order/ Proceedings	Order with signatures of Judge, and that of parties or counsel, where necessary.
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11.03.2025 Malik Shafqat Raza Thaheem, Advocate for the petitioner in Crl. Misc. No.10010-B of 2024.
Rana Umar Daraz Khan, Advocate for the petitioner in Crl. Misc. No.1060-B of 2025.
Mr. Waheed Rafiq, Deputy District Public Prosecutor with Asghar, SI.
Mr. Fahad Aziz Khan Punian, Advocate for the complainant.

This single order shall dispose of titled petitions filed under Section 497 Cr.P.C., by the petitioners seeking post arrest bail in case FIR No.349 dated 03.06.2024 registered under Sections 302/324/148/149-PPC at Police Station Nawan Shahr, District Khanewal.

2. Petitioners were amongst 04 nominated and 02 unknown accused who assaulted upon Shaukat Hayat, brother of the complainant. Fire made by co-accused Nasir with repeater missed the target, whereas role given to the petitioners and co-accused Yasin was of causing injuries with clubs. Yasin’s blow hit on the head and right elbow whereas Muhammad Ramzan and Jahangir, petitioners hit on left claw and elbow & on his right ankle and knee, respectively; then all the accused committed torture upon said Shaukat Hayat.

3. Learned counsel (s) for the petitioners contend that it was an occurrence of 01.06.2024 at 09:30 a.m. which was reported to the police with a delay of two days on 03.06.2024 whereas injured died on 05.06.2024 and neither the investigating officer has recorded the statement of Shaukat Hayat nor the doctor has observed any internal

injury on the person of deceased as reflected from the information contained in columns of postmortem report relating to ‘Cranium and spinal cord’, thorax, abdomen and ‘Upper & lower limbs’ that organs are healthy or “No Abnormality Detected” (NAD). So much so, injury attributed to Yasin co-accused on the head was not found at said locale during postmortem examination of deceased. Further state though there were 8 injuries on upper and lower limbs of the deceased but doctor has declared the cause of death as under;

“After performing autopsy and noting all the findings, I am of the opinion that the Death occur due to multiple injuries during fight as per ward report which lead to his altered GCS result in serious condition was refer to NHM there he could not revive and death due to Cardiopulmonary arrest.”

4. Learned counsel for the complainant opposed the bail on the grounds that deceased was 57 years of age who fell fainted by a furious assault by the petitioners and other accused; Initial Medicolegal Officer (IMLO) had also observed that “*as the patient was losing his conscious level due to multiple injuries, so he was shifted to NHM (Nishtar Hospital Multan).*” Further states that due to such injuries his Glasgow Coma Scale (GCS) went down which resulted in cardiopulmonary arrest, therefore, petitioners cannot escape from the criminal liability nor they be let off.

5. Heard; record perused.

6. Though doctor has observed lacerated injuries on the person of deceased without commenting upon its fatality yet has not given any exact reason of cardiopulmonary arrest or cardiac arrest. There are multiple reasons for altered GCS and cardiopulmonary arrest can also be caused by a variety of factors, primarily stemming from heart problems or other medical conditions, including arrhythmias, heart attacks, and structural heart issues. As per medical literature, Cardiac Causes include;

(i) ‘Arrhythmias’ which means irregular heart rhythms, especially ventricular fibrillation (ii) Coronary Artery Disease (CAD), blockages in the arteries supplying blood to the heart can lead to a heart attack, which can trigger cardiac arrest (iii) Heart Attack, can disrupt the heart's electrical system and lead to a sudden cardiac arrest (iv) Heart Failure, when the heart cannot pump blood effectively, it can lead to a weakened state and potentially cardiac arrest (v) Enlarged Heart (Cardiomyopathy), thickening or stretching of the heart muscle can

disrupt its ability to function properly and lead to arrhythmias (vi) Valvular Heart Disease, problems with the heart valves can strain the heart and increase the risk of arrhythmias and cardiac arrest (vii) Congenital Heart Conditions, heart defects present at birth can increase the risk of cardiac arrest (viii) Electrical System Malfunction, problems with the heart's electrical system can cause abnormal heart rhythms (ix) Scarring of the heart tissue, this can be caused by a prior heart attack or other causes (x) Thickened heart muscle, this can be caused by high blood pressure, heart valve disease, or other causes (xi) Heart medications, some heart medications can cause arrhythmias that cause sudden cardiac arrest.

Non-Cardiac Causes are as under;

(i) Respiratory Problems (ii) Trauma, severe injuries, especially to the chest, can disrupt the heart's function and lead to cardiac arrest. (iii) Electrocutation, exposure to high voltage electricity can cause a sudden disruption of the heart's electrical system (iv) Drug Overdose (v) Severe Illness, conditions like sepsis or blood loss can lead to cardiac arrest (vi) Electrolyte Imbalances, abnormal levels of potassium, magnesium, or other electrolytes can disrupt the heart's function (vii) Hypothermia, extremely low body temperature can cause the heart to stop beating (viii) Intracranial Hemorrhage, bleeding in the brain can disrupt the body's ability to function properly, including the heart (ix) Pulmonary Embolism, a blood clot in the lungs can disrupt blood flow to the heart and cause cardiac arrest (x) Pneumothorax, a collapsed lung can disrupt the body's ability to get oxygen, which can lead to cardiac arrest (xi) Risk Factors like family History of Heart Disease or Cardiac Arrest or of heart problems (xii) High Blood Pressure (xiii) High Cholesterol, can lead to the buildup of plaque in the arteries, which can increase the risk of heart attack and cardiac arrest (xiv) Smoking, damages the heart and blood vessels (xv) Obesity (xvi) Diabetes (xvii) Lack of Physical Activity (xviii) Alcohol or Drug Abuse, substance abuse can damage the heart and increase the risk of cardiac arrest.

From the above information, it is reflected that the multiple reasons for cardiopulmonary arrest do include trauma/injuries; therefore, it is essential to see what is Glasgow Coma Scale and effect of body trauma on such Scale.

7. The Glasgow Coma Scale (GCS) is a tool used to assess a patient's level of consciousness by evaluating their eye, verbal, and motor responses, with scores ranging from 3 to 15, where 3 indicates a comatose state and 15 represents normal consciousness. The GCS was developed in 1974 by experts at the University of Glasgow in Scotland. It's a widely used tool for measuring consciousness and coma. It assesses a person's ability to perform eye movements, speak, and move their body. The Glasgow Coma Scale has three categories that apply to a neurological examination. Most of them

apply to the brain itself, but some can also involve spinal cord and nerves throughout the body. The GCS is measured with the help of followings;

- Eye response:** This relates to how awake and alert you are.
- Motor response:** This part is about how well your brain can control muscle movement. It can also show if there are any issues with the connections between your brain and the rest of your body.
- Verbal response:** This tests how well certain brain abilities work, including thinking, memory, attention span and awareness of your surroundings.

The scores for each category (E, V, M) are added together to give a total GCS score. The GCS score should be recorded individually, for example, E2V3M4 results in a GCS score of 9. The GCS is used to objectively describe the extent of impaired consciousness in all types of acute medical and trauma patients. In the medical context, consciousness has three requirements; **(i) Awake:** this includes whether or not you have the ability to wake up because of voice or touch. That’s what makes a coma different from just being asleep **(ii) Alert:** this is how responsive you are to people talking to you and if you’re able to understand what’s happening in your immediate surroundings **(iii) Oriented:** this means you know who you are, where you’re at, what day it is and other details related to the here and now.

8. A declining Glasgow Coma Scale (GCS) score, which measures a person's level of consciousness, typically indicates a worsening neurological condition, potentially due to factors like head injury, stroke, or other brain-related issues. Factors that can cause a GCS score to decrease are as under;

- (i) Traumatic Brain Injury (TBI):**
Head injuries, including concussions and more severe trauma, can lead to swelling, bleeding, or damage to brain tissue, all of which can impair consciousness and lower the GCS score.
- (ii) Cerebrovascular Accidents (Stroke):**
Strokes, whether ischemic (due to blocked blood flow) or hemorrhagic (due to bleeding), can disrupt brain function and cause a decline in GCS.
- (iii) Intracranial Infections or Abscesses:**
Infections or abscesses within the brain can lead to inflammation and pressure, which can affect brain function and consciousness.

(iv) Other Neurological Conditions:

Conditions like epilepsy, poisoning, or even certain psychiatric disorders can also lead to a reduced level of consciousness and a lower GCS score.

(v) Physiological Derangements:

Factors like hypoxia (low blood oxygen), shock, or hypoglycemia (low blood sugar) can impair brain function and lower the GCS score.

(vi) Medications and Intoxication: Certain medications, including sedatives, or drug and alcohol intoxication can also depress consciousness and affect the GCS score.

(vii) Intubation: If a patient is intubated and unable to speak, they are evaluated only on the motor and eye-opening response and the suffix T is added to their score to indicate intubation.

(viii) Pre-existing conditions:

Pre-existing conditions like dementia, speech and hearing impairment can also affect the GCS score.

GCS usually decreases due to head injuries, including concussions and more severe trauma, which can lead to swelling, bleeding, or damage to brain tissue, but in this case no head injury was observed by the doctor. Thus, the GCS is not a perfect tool which can be influenced by various factors, so it should be used in conjunction with other clinical assessments which the doctor has not mentioned in detail, leaving the subject to be scrutinized through examination of doctor as witness during the trial.

9. It has been observed that Shaukat Hayat when injured had himself spoken to IMLO and gave history of fight; his name is also mentioned in the corner of MLR relating to “Halfia Bian”, statement on oath showing consent of injured for his first medical examination, but neither the investigating officer has recorded his statement, nor the doctor had ascertained that who actually caused him injuries, which could have been a best res gestae evidence and a support for involvement of present petitioners in the capacity of their individual liability. A case reported as “MUHAMMAD ATIF NAVEED and another Versus The STATE” (2024 P Cr. LJ 1421) is referred in this respect. It has further been observed that as per history given by Shaukat Hayat before the IMLO on 01.06.2024, he was beaten by 4 nominated and 4 unknown-accused yet without specifying the names of any of the accused persons. Even otherwise by then no FIR was registered and there was no story of prosecution so as to stretch it in

conjunction with history given by him. Later story came on the record through registration of FIR on 03.06.2024 which was at variance with respect of number of accused, therefore, factor of consultation and concoction seems imbedded in the prosecution case. Thus, all such facts would better be thrashed during the trial.

10. From the above discussion, it is clear that there are multiple reasons for cardiopulmonary arrest and decreasing of Glasgow Coma Scale (GCS); therefore, until the doctor is examined during the trial, to ascertain the exact reason for death of deceased, the individual or joint criminal liability of the petitioners cannot be assessed tentatively at this stage of the proceedings which function shall be performed by learned trial Court.

11. At present, sufficient material is not available on record to connect the petitioners with the liability that due to their act, deceased had breathed his last. The above situation provides a premium to the petitioners to seek bail on the touchstone of further inquiry which is very much available in this case. Thus, petitioners have made out a case for further inquiry. Reliance in this respect is placed on cases reported as “MAZHAR ALI Versus The STATE and another” (2025 SCMR 318), “ZEESHAN Versus The STATE and another” (2024 SCMR 1716), “Mst. ISHRAT BIBI Versus The STATE through Prosecutor General, Punjab and another” (2024 SCMR 1528), “SAEED AHMED and another Versus The STATE” (PLD 2024 Supreme Court 1241), “SALMAN ZAHID Versus The STATE through P.G. Sindh” (2023 SCMR 1140) “GUL MUHAMMAD Versus The STATE” (2023 SCMR 857) & “MUHAMMAD NAEEM HASSAN Versus The STATE through P.G., Punjab and another” (2022 SCMR 523). Jahangir petitioner is behind the bars since 07.07.2024 whereas Muhammad Ramzan petitioner is behind the bars since 17.07.2024, investigation is complete and persons of the petitioners are not required for further investigation; thus, keeping them behind the bars for indefinite period would not serve any useful purpose to the prosecution.

12. In view of what has been discussed above, titled petitions are **allowed** and the petitioners are admitted to bail subject to furnishing bail bonds in the sum of **Rs.500,000/-** each with one surety each in the like amount to the satisfaction of the trial Court. However, above assessment of this Court is tentative in nature which shall not prejudice the case of prosecution during the trial.

(MUHAMMAD AMJAD RAFIQ)
JUDGE

Approved for reporting:

Judge

Signed on

*M. Azhar**